

CANONICAL LESSON

Trust the Health Indicator Only After Testing Its Denominator

Decision-centred foundations for accountable sector intelligence

Production Unit	PU-B07-C26
Notebook	V7_B_N06
Status	Controlled educational draft v0.1
Data boundary	Synthetic instructional data only

Central decision: Trust a health indicator only after testing its numerator, denominator, target population, reference period, coverage, and revision status.

Learning outcomes

Why this matters

A plausible rate can mislead when its denominator excludes, duplicates, or misclassifies the population at risk.

The analytical workflow is useful only when evidence, uncertainty, authority, and action are connected explicitly. The model supports professional judgement; it does not replace accountable institutional authority.

Worked cross-sector example

A maternal-care rate uses eligible pregnancies rather than all women as its denominator.

Boundary: The example supports learning and review. It does not constitute an official statistic, eligibility determination, enforcement action, clinical judgement, or operational authorization.

Practice and reflection

- 1.
- Detect duplicates.
- 2.
- Reconcile changing catchments.

- 3.
- Explain why missing is not zero.

Answer key

- 1.
- Use event/report keys, timestamps, source identifiers, and duplicate-resolution rules.
- 2.
- Version facility catchments and population denominators by period.
- 3.
- Zero asserts no events; missing means no usable observation and requires follow-up or principled estimation with uncertainty.

Operational readiness checklist

- Purpose and decision owner are named.
- Evidence definitions, provenance, and lawful access are documented.
- Uncertainty and abstention are visible.
- Human review, escalation, monitoring, incident response, and retirement are assigned.
- Equity, accessibility, security, and sector-specific approval are complete.